PharmEasy Labs

Report for Sarika Singh(46Y/F)

Tests asked Comprehensive Full Body Checkup With Vitamin D And B12 - 3, Ferr

+ 1 Others

Test date 10 Aug 2025 **Report status** Complete Report

REPORT VERIFIED VERIFIED BY DOCTOR

6 STEP quality control to ensure 100% report accuracy

- Qualified and trained technicians
- Temperature-controlled containers to store samples
- Strict quality checks on samples before processing
- Regular monitoring of lab analyzers by experts
- Assured machine inspection on a daily basis
- Verified reports by qualified pathologists

25+ Years of Trust & Experience

100+ Crore Samples Processed

PharmEasy Labs

NAME	:	SARIKA SINGH(46Y/F)
REF. BY	:	SELF
TEST ASKED	:	Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERR,SERUM ELECTROLYTES

Home Collection

A4-703 PARAMOUNT PILATUS 80 FT ROAD
AREKERE L BANGALORE B T M LAYOUT
BENGALURU

Report Availability Summary

Note: Please refer to the table below for status of your tests.

13 Ready | **0** Ready with Cancellation | **0** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
SERUM ELECTROLYTES	Ready
FERRITIN	Ready
Comprehensive Full Body Checkup With Vitamin D and B12 - 3	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
25-OH VITAMIN D (TOTAL)	Ready
IRON	Ready
VITAMIN B-12	Ready
COMPLETE URINE ANALYSIS	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Ready
LIVER FUNCTION TESTS	Ready
KIDPRO	Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready

PharmEasy Labs

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TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	96.57	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.

Method:- GOD-PAP METHOD

Sample Collected on (SCT)	:	10 Aug 2025 08:37
Sample Received on (SRT)	:	10 Aug 2025 11:56
Report Released on (RRT)	:	10 Aug 2025 12:42
Sample Type	:	FLUORIDE PLASMA
Labcode	:	1008069285/DG007
Barcode	:	EI201281

Deshpande

Dr.Shalaka MD(Path)

Dr.Ishant MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: 1st Floor, 889 HSR layout Sector-7 (BDA), No 1159, Bangalore - 560102
7022 000 900

PharmEasy Labs

NAME : SARIKA SINGH(46Y/F)
REF. BY : SELF
TEST ASKED : Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERRITIN,SERUM ELECTROLYTES

Home Collection
A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
Complete Urinogram				
Physical Examination				
VOLUME	Visual Determination	3	mL	-
COLOUR	Visual Determination	PALE YELLOW	-	Pale Yellow
APPEARANCE	Visual Determination	CLEAR	-	Clear
SPECIFIC GRAVITY	pKa change	1.02	-	1.003-1.030
PH	pH indicator	5.5	-	5-8
Chemical Examination				
URINARY PROTEIN	PEI	ABSENT	mg/dL	Absent
URINARY GLUCOSE	GOD-POD	ABSENT	mg/dL	Absent
URINE KETONE	Nitroprusside	ABSENT	mg/dL	Absent
URINARY BILIRUBIN	Diazo coupling	ABSENT	mg/dL	Absent
UROBILINOGEN	Diazo coupling	Normal	mg/dL	<=0.2
BILE SALT	Hays sulphur	ABSENT	-	Absent
BILE PIGMENT	Ehrlich reaction	ABSENT	-	Absent
URINE BLOOD	Peroxidase reaction	ABSENT	-	Absent
NITRITE	Diazo coupling	ABSENT	-	Absent
LEUCOCYTE ESTERASE	Esterase reaction	ABSENT	-	Absent
Microscopic Examination				
MUCUS	Microscopy	ABSENT	-	Absent
RED BLOOD CELLS	Microscopy	ABSENT	cells/HPF	0-5
URINARY LEUCOCYTES (PUS CELLS)	Microscopy	ABSENT	cells/HPF	0-5
EPITHELIAL CELLS	**Microscopy**	**7**	**cells/HPF**	**0-5**
CASTS	Microscopy	ABSENT	-	Absent
CRYSTALS	Microscopy	ABSENT	-	Absent
BACTERIA	Microscopy	ABSENT	-	Absent
YEAST	Microscopy	ABSENT	-	Absent
PARASITE	Microscopy	ABSENT	-	Absent

(Reference : *PEI - Protein error of indicator, *GOD-POD - Glucose oxidase-peroxidase**)**

Sample Collected on (SCT) : 10 Aug 2025 08:37
Sample Received on (SRT) : 10 Aug 2025 12:04
Report Released on (RRT) : 10 Aug 2025 13:25
Sample Type : URINE
Labcode : 1008069971/DG007
Barcode : EC517218

Deshpande
Dr.Shalaka MD(Path)

Dr.Ishant MD(Path)

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NAME : SARIKA SINGH(46Y/F)
REF. BY : SELF
TEST ASKED : Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERRITIN,SERUM ELECTROLYTES

Home Collection
A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	**H.P.L.C**	**5.9**	**%**

Bio. Ref. Interval. :

As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	**CALCULATED**	**123**	**mg/dL**

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 10 Aug 2025 08:37
Sample Received on (SRT) : 10 Aug 2025 12:08
Report Released on (RRT) : 10 Aug 2025 14:07
Sample Type : EDTA Whole Blood
Labcode : 1008070577/DG007
Barcode : EI632450

Dr.Shalaka MD(Path)

Dr.Ishant MD(Path)

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AREKERE L BANGALORE B T M LAYOUT
BENGALURU

TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	12.6	g/dL	12.0-15.0
Hematocrit (PCV)	CPH Detection	41.1	%	36.0-46.0
Total RBC	**HF & EI**	**5.03**	**X 10^6/µL**	**3.8-4.8**
Mean Corpuscular Volume (MCV)	**Calculated**	**81.7**	**fL**	**83.0-101.0**
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**25**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	**Calculated**	**30.7**	**g/dL**	**31.5-34.5**
Red Cell Distribution Width - SD (RDW-SD)	Calculated	42	fL	39.0-46.0
Red Cell Distribution Width (RDW - CV)	**Calculated**	**14.5**	**%**	**11.6-14.0**
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	235.5	-	*Refer Note below
MENTZER INDEX	Calculated	16.2	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	8.59	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	59.7	%	40-80
Lymphocytes Percentage	Flow Cytometry	30.3	%	20-40
Monocytes Percentage	Flow Cytometry	4.8	%	2-10
Eosinophils Percentage	Flow Cytometry	4.7	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.13	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.6	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.41	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.04	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.4	X 10^3 / µL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	363	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	9.3	fL	6.5-12
Platelet Distribution Width (PDW)	**Calculated**	**9.2**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**19.5**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.34	%	0.19-0.39

Remarks : Alert!!! Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

Sample Collected on (SCT)	:	10 Aug 2025 08:37
Sample Received on (SRT)	:	10 Aug 2025 12:08
Report Released on (RRT)	:	10 Aug 2025 14:07
Sample Type	:	EDTA Whole Blood
Labcode	:	1008070577/DG007
Barcode	:	EI632450

Dr.Shalaka MD(Path)

Dr.Ishant MD(Path)

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PharmEasy Labs

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Home Collection
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AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.M.I.A**	**18.5**	**ng/mL**

Bio. Ref. Interval. :-

DEFICIENCY : <10 ng/ml
INSUFFICIENCY : 10 - 30 ng/ml
SUFFICIENCY : 30 - 100 ng/ml
TOXICITY : >100 ng/ml

Please correlate with clinical conditions.

Method:- Fully Automated Chemi Luminescent Microparticle Immunoassay

Sample Collected on (SCT)	:	10 Aug 2025 08:37
Sample Received on (SRT)	:	10 Aug 2025 12:07
Report Released on (RRT)	:	10 Aug 2025 16:31
Sample Type	:	SERUM
Labcode	:	1008070483/DG007
Barcode	:	EH389542

Deshpande
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REF. BY : SELF
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Home Collection
A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	57	µg/dL

Bio. Ref. Interval. :

Male : 65 - 175
Female : 50 - 170
Method : Ferrozine method without deproteinization

FERRITIN	C.M.I.A	27.8	ng/mL

Bio. Ref. Interval. :

Men: 21.81 - 274.66 ng/ml
Women: 4.63 - 204.00 ng/ml
Method : Fully Automated Chemi Luminescent Microparticle Immunoassay

Please correlate with clinical conditions.

Sample Collected on (SCT) : 10 Aug 2025 08:37
Sample Received on (SRT) : 10 Aug 2025 12:07
Report Released on (RRT) : 10 Aug 2025 16:31
Sample Type : SERUM
Labcode : 1008070483/DG007
Barcode : EH389542

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PharmEasy Labs

NAME	: SARIKA SINGH(46Y/F)	**Home Collection**
REF. BY	: SELF	A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU
TEST ASKED	: Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERRITIN,SERUM ELECTROLYTES	

TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	**C.L.I.A**	**196**	**pg/mL**

Bio. Ref. Interval. :-

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

Please correlate with clinical conditions.

Method:- COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

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Report Released on (RRT)	: 10 Aug 2025 16:31
Sample Type	: SERUM
Labcode	: 1008070483/DG007
Barcode	: EH389542

Deshpande

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	**PHOTOMETRY**	**208**	**mg/dL**	**< 200**
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	44	mg/dL	40-60
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**140.5**	**mg/dL**	**< 100**
TRIGLYCERIDES	PHOTOMETRY	107	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	4.7	Ratio	3 - 5
TRIG / HDL RATIO	CALCULATED	2.42	Ratio	< 3.12
LDL / HDL RATIO	CALCULATED	3.2	Ratio	1.5-3.5
HDL / LDL RATIO	**CALCULATED**	**0.32**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	**CALCULATED**	**164.1**	**mg/dL**	**< 160**
VLDL CHOLESTEROL	CALCULATED	21.44	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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Report Released on (RRT) : 10 Aug 2025 16:31
Sample Type : SERUM
Labcode : 1008070483/DG007
Barcode : EH389542

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	66.7	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.85	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.14	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.71	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	21.4	U/L	< 38
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	17.9	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	19.6	U/L	< 34
SGOT / SGPT RATIO	CALCULATED	0.91	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	6.91	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	3.88	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.03	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.28	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS
SODIUM	I.S.E - INDIRECT	138	mmol/L

Bio. Ref. Interval. :

Adults: 136-145 mmol/l

Method : ION SELECTIVE ELECTRODE - INDIRECT

POTASSIUM	I.S.E - INDIRECT	4.5	mmol/L

Bio. Ref. Interval. :

ADULTS: 3.5-5.1 MMOL/L

Clinical Significance :
An abnormal increase in potassium (hyperkalemia)can profoundly affect the nervous system and increase the chance of irregular heartbeats (arrhythmias), which ,when extreme ,can be fatal. The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Potassium in a given specimen may vary due to differences in assay methods, calibration and reagent specificity.

Method : ION SELECTIVE ELECTRODE - INDIRECT

CHLORIDE	I.S.E - INDIRECT	105	mmol/L

Bio. Ref. Interval. :

ADULTS: 98-107 MMOL/L

Clinical Significance :
An increased level of blood chloride (called hyperchloremia) usually indicates dehydration, but can also occur with other problems that cause high blood sodium, such as Cushing syndrome or kidney disease. Hyperchloremia also occurs when too much base is lost from the body (producing metabolic acidosis) or when a person hyperventilates (causing respiratory alkalosis). A decreased level of blood chloride (called hypochloremia) occurs with any disorder that causes low blood sodium. Hypochloremia also occurs with congestive heart failure, prolonged vomiting or gastric suction, Addison disease, emphysema or other chronic lung diseases (causing respiratory acidosis), and with loss of acid from the body (called metabolic alkalosis).

Method : ION SELECTIVE ELECTRODE - INDIRECT

Please correlate with clinical conditions.

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A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	9.62	mg/dL	7.94 - 20.07
CREATININE - SERUM	PHOTOMETRY	0.71	mg/dL	0.55-1.02
BUN / Sr.CREATININE RATIO	CALCULATED	13.55	Ratio	9:1-23:1
UREA (CALCULATED)	CALCULATED	20.59	mg/dL	Adult : 17-43
UREA / SR.CREATININE RATIO	CALCULATED	29	Ratio	< 52
CALCIUM	PHOTOMETRY	8.83	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	5.2	mg/dL	3.2 - 6.1

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

Sample Collected on (SCT) : 10 Aug 2025 08:37
Sample Received on (SRT) : 10 Aug 2025 12:07
Report Released on (RRT) : 10 Aug 2025 16:31
Sample Type : SERUM
Labcode : 1008070483/DG007
Barcode : EH389542

Dr.Shalaka MD(Path)

Dr.Ishant MD(Path)

Note:- Underlined values are Critical Values, Clinician's attention required.

Clinically Tested by :Thyrocare Technologies Ltd

PharmEasy Labs
Registered Address: DOCON TECHNOLOGIES PRIVATE LIMITED, Office No 208, 209 & 210 Second floor, A wing, 'Dattani Plaza', Near East West Industrial Estate, Safed Pool, Saki Naka, Andheri (East), Mumbai - 400072
Lab Address: 1st Floor, 889 HSR layout Sector-7 (BDA), No 1159, Bangalore - 560102
7022 000 900

PharmEasy Labs

NAME	:	SARIKA SINGH(46Y/F)
REF. BY	:	SELF
TEST ASKED	:	Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERRITIN,SERUM ELECTROLYTES

Home Collection
A4-703 PARAMOUNT PILATUS 80 FT ROAD AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	**C.M.I.A**	**57**	**ng/dL**	**58-159**
TOTAL THYROXINE (T4)	C.M.I.A	9.11	µg/dL	4.87-11.72
TSH - ULTRASENSITIVE	C.M.I.A	4.19	µIU/mL	0.35-4.94

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3 - Fully Automated Chemi Luminescent Microparticle Immunoassay
T4 - Fully Automated Chemi Luminescent Microparticle Immunoassay
USTSH - Fully Automated Chemi Luminescent Microparticle Immunoassay

Pregnancy reference ranges for TSH/USTSH :

Trimester	T3 (ng/dl)	T4 (µg/dl)	TSH/USTSH (µIU/ml)
1st	83.9-196.6	4.4-11.5	0.1-2.5
2nd	86.1-217.4	4.9-12.2	0.2-3.0
3rd	79.9-186	5.1-13.2	0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243
2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer :

Results should always be interpreted using the reference range provided by the laboratory that performed the test.
Different laboratories do tests using different technologies, methods and using different reagents which may cause difference In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports.
To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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Clinically Tested by :Thyrocare Technologies Ltd

NAME : SARIKA SINGH(46Y/F)
REF. BY : SELF
TEST ASKED : Comprehensive Full Body Checkup With Vitamin D and B12 - 3,FERRITIN,SERUM ELECTROLYTES

Home Collection
A4-703 PARAMOUNT PILATUS 80 FT ROAD
AREKERE L BANGALORE B T M LAYOUT BENGALURU

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	106	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

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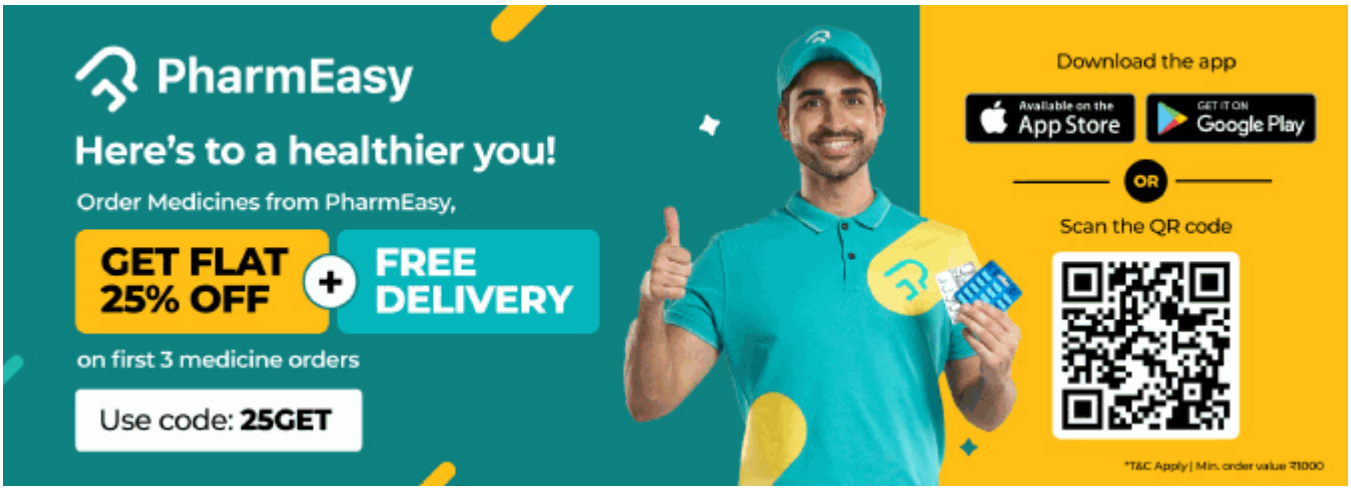
CONDITIONS OF REPORTING

- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Docon Technologies Private Limited,Thyrocare Technologies Limited and its employees/representatives do not assume any liability,responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report.

EXPLANATIONS

- **Name -** The name is as declared by the client and recorded by the personnel who collected the specimen.
- **Ref.By -** The name of the doctor who has recommended testing as declared by the client.
- **Labcode -** This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode -** This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCT -** Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT -** Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT -** Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range -** Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- For suggestions, complaints or feedback, write to us at grievance-office@docon.co.in or call us on 7022000900.